

2.5 Clinical Overview

AMLOZA (Amlodipine Besylate Tablets, USP) 5 mg / 10 mg — ANDA

As an ANDA, this application does not include independently generated clinical efficacy or safety studies. Clinical efficacy and safety for the amlodipine besylate active ingredient, at the proposed strengths and for the proposed indications, are established by the approved labeling of the RLD (Norvasc®). The clinical bridge between the RLD and AMLOZA is provided by the bioequivalence data presented in Module 5.

2.7.1 Clinical Pharmacology

Amlodipine's mechanism of action (inhibition of calcium ion influx across cardiac and vascular smooth muscle cell membranes, producing coronary and peripheral vasodilation) and its established pharmacokinetic profile (slow absorption, T_{max} 6–12 hours, extensive hepatic metabolism, elimination half-life 30–50 hours supporting once-daily dosing) are as reported in the RLD's approved labeling, reproduced for reference in Module 1.11 (Full Prescribing Information). No independent clinical pharmacology data were generated for AMLOZA; the profile is relied upon from the RLD since AMLOZA contains the same active moiety at the same strengths.

2.7.2 Summary of Clinical Efficacy

Not independently established for this ANDA. Efficacy for the treatment of hypertension and chronic stable/vasospastic angina is relied upon from the RLD's approved labeling and the extensive published clinical literature on amlodipine besylate, consistent with the generic drug approval pathway.

2.7.3 Summary of Clinical Safety

Not independently established for this ANDA beyond the safety monitoring conducted within the bioequivalence study itself (Module 5). The established adverse reaction profile, warnings, and contraindications (see Module 1.11, Sections 4–8) are relied upon from the RLD's approved labeling.

2.7.4 Benefit-Risk Conclusion

The benefit-risk profile of AMLOZA would be expected to be comparable to that of the RLD (Norvasc®), based on demonstrated bioequivalence (Module 5), identical active ingredient and strengths, compendially controlled excipients, and reliance on FDA's prior finding of safety and effectiveness for the reference product.